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When Help Is Harm: Health, Lookism and Self-Improvement in the Manosphere

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ABSTRACT

How do online communities impact men's health? How does hegemonic masculinity harm men's health? In this paper, we analyse an online looksmaxxing community that receives 6 million unique visitors per month and is aimed at men seeking to improve their appearance. We qualitatively analysed 8072 discussion board comments in total. Our findings first demonstrate how users apply a hegemonic masculine gaze as they critique men's bodies (e.g., facial asymmetries and brow ridges). Second, we show how the community encourages men to substantially alter their bodies (e.g., with leg lengthening surgeries or by mewing). Third, we argue that the community subjects users to masculine demoralisation, wherein they are seen as failed men and encouraged to self-harm. Drawing on masculinities theory, we argue that looksmaxxing and similar self-improvement communities harm the health of the men who participate in them.

1 | Introduction

How do online communities impact men's health? How does hegemonic masculinity harm men's health? In this paper, we analyse a popular online looksmaxxing community for men that focuses on rating physical appearance and providing self-improvement recommendations (e.g., surgery). 'Looksmaxxing' means maximising one's physical appearance to attract partners and receive the social benefits that accompany being attractive. We argue that looksmaxxing harms the physical and mental health of the men and boys who participate. As looksmaxxing is growing in popularity and promoted on many social media platforms (Notopoulos 2023), we propose that looksmaxxing is an emerging health and social issue.

Looksmaxxing communities are part of the manosphere, a group of male-oriented websites that share a 'red pill' ideology. Red pill communities support misogyny, embrace traditional gender roles, critique feminism and suggest that men are victims of misandry (e.g., Dignam and Rohlinger 2019; Van Valkenburgh 2021; Halpin 2022; Halpin et al. 2025). These communities have been extensively analysed in relation to misogyny and violence (e.g., Gosse, Halpin and Maguire 2024). In this paper, we examine how they also harm the men who participate in them.

Our study develops concepts, analyzes the performance of gender and health, and articulates the consequences of self-improvement practices in the manosphere. First, we argue that users apply a hegemonic masculine gaze as they critique one another's bodies (e.g., in relation to height or musculature). This gaze demeans, body shames, and humiliates users under the auspices of 'objective' masculine advice. Second, we demonstrate how ratings are tied to self-improvement and

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health/medical interventions, such as mewing (pushing the tongue against the roof of the mouth to achieve a more masculine jaw) and surgeries (e.g., leg-lengthening). Third, despite being positioned as a support community, we contend that the looksmaxxing community subjects men to masculine demoralisation by denigrating users' appearances and encouraging them to complete suicide. As we conclude, the community buttresses hegemonic masculinity and male supremacy while harming the health of men and boys who participate.

2 | Background: Masculinity, Lookism and Health

Our study builds on masculinities theory and research. Connell's (1995) masculinities theory identifies hegemonic masculinity as a culturally valued set of masculine practices that maintains the subjugation of women and benefits from the subordination of other men. Connell further details masculine performances that passively benefit from patriarchy (complicit masculinity), performances that are excluded from masculinity (subordinate masculinity) and the performances of socially marginalised men (marginalised masculinity).

Building on Connell's work, Courtenay (2000a, 2000b) provides a framework arguing that health is one means by which men 'do gender'—or how they engage in or avoid health practices that adhere to socially constructed notions of what is 'masculine'. Courtenay argues that to enact hegemonic notions of masculinity, men must adhere to health beliefs and behaviours consistent with masculine norms and reject those associated with femininity. As he argues, the resources available to men to perform masculinity are largely unhealthy, leading men to engage in risky behaviours while avoiding health-promoting behaviours.

Research both supports and contradicts Courtenay's framework. Supportive studies suggest that hegemonic masculinity can harm health and that, compared to women, men are less likely to engage in preventative care, seek medical help and are more likely to suppress problems (e.g., Galdas, Cheater, and Marshall 2005; Ratcliffe, Kanaan, and Galdas 2023; River and Flood 2021; Johnson et al. 2012; Springer and Mouzon 2011). Other studies conversely suggest that men deviate from hegemonic masculinity by aligning health with masculinity (Quaglia 2020; Simpson and Richards 2019). Similarly, Watson (2000) suggests that research can reduce men's health behaviours to 'risk-taking' practices, which obscures their lived experiences and their focus on maintaining an 'everyday body'. Work on exercise and steroid use further complicates Courtenay's framework, as men can consume drugs that can harm their health yet they can also frame weightlifting as part of the pursuit of a 'healthy', lean and masculine body (e.g., Denham 2008; Jordan-Young and Karkazis 2019; L. Monaghan 1999). Courtenay's framework is also contrastive to work arguing that masculinity can be medicalised (Bell 2016). Here, instead of avoiding medicine, men might use it to buttress their gendered performance, such as seeking medication to improve the quality of erections (Rosenfeld and Faircloth 2006). In addition to developing novel concepts for analysing looksmaxxing, masculinity, and health, we forward revisions to Courtenay's (2000a) framework in our discussion.

We combine this work on masculinity and health with research on lookism and body image. Lookism is a form of discrimination and prejudice that favours attractive people (Dion, Berscheid, and Walster 1972). Lookism has been connected to several negative outcomes (e.g., reduced wages), including poor health outcomes (e.g., Monk, Esposito, and Lee 2021). In relation to lookism, studies demonstrate that social media can negatively impact the body image and mental health of both men and women (e.g., Perloff 2014; Maslen and Lupton 2019; Filice et al. 2020; Fixsen et al. 2023; Prichard et al. 2020; Dahlgren et al. 2024). Similarly, research indicates that men judge their bodies in relation to masculine norms while feeling pressured to remain nonchalant about their efforts to improve their appearances (Grogan and Richards 2002; Norman 2011; Simpson and Adams 2019).

We combine masculinities and lookism research by describing the hegemonic masculine gaze and demonstrating how it has consequences for the health of men. We argue this gaze becomes medicalised, as men report pursuing behavioural and surgical interventions to 'improve' their appearance. Finally, we contend that men in the looksmaxxing community experience masculine demoralisation as users consider self-harm because they view themselves as 'failed men'.

3 | Data and Methods

The community we study is advertised as being for men who want to improve their looks and enjoy the perceived social advantages associated with attractiveness. It receives millions of unique visitors per month, hosts tens of thousands of members, and contains millions of comments. Although there is no demographic data about users, we observed individuals ranging from teens to middle-aged, both straight and gay users, and users self-identifying with a variety of racial/ethnic categories.

Building on similar online investigations (e.g., Halpin 2022; Hine 2015; Solea and Sugiura 2023), in 2023, we collected data from the two most popular forums on a looksmaxxing website. First, we analysed 5327 comments on a looksmaxxing discussion board, where users discuss fitness, style, and surgery, with the goal of self-improvement. Second, we analysed 2745 comments on a ratings discussion board, where users post their pictures and ask for ratings and advice from other members. The site we analyse is public; however, we do not name the site in this article or provide usernames in case these are shared with different people on other sites. We likewise elected not to analyse photos. We use verbatim quotes because modifying quotes can alter the meaning or interpretation of statements. We can provide the name of the community we analyse upon request so other researchers can interrogate our claims about the community. This analysis is part of a larger project that received ethics board approval (#2019-4856) from Dalhousie University. Initial approval was granted in 2019.

We analyse online communities because our online lives are increasingly consequential. We specifically examine this looksmaxxing community for three reasons. First, it is a discussion board, which permits analysis of how looksmaxxing procedures

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are discussed, debated, and promoted by members (see also Hine 2015). Second, such interactional data is not accessible via alternative approaches (e.g., interviews and surveys), although these other methods are also of value. Third, looksmaxxing is tightly coupled to online activity, with its growing popularity driven by discussion boards and social media (Notopoulos 2023).

To focus analyses on topics that were engaging and being actively discussed by users, we sampled threads that received at least 100 replies and at least one reply in 6 months before the start of our data collection period in 2023. We then pilot-tested our codebook on a subsample of data to ensure we were applying codes in a similar manner. Once pilot testing was complete, we formalised our codebook with the names of codes, examples, and operational definitions. The first and second authors completed all coding. We used the Nvivo software to code and organise data.

Our analysis is guided by analytic abduction (Tavory and Timmermans 2014), which focuses on how findings might extend, challenge, or refine preexisting theories or concepts. In this paper, we focus on how our findings are relevant to theories of masculinity and health (e.g., Connell 1995; Courtenay 2000a, 2000b). For instance, we develop the concept of masculine demoralisation to extend masculinity theory and suggest revisions to theoretical work on masculinity and health in our discussion.

Our study has limits. First, we assume that users' comments are authentic and reflect their experiences. However, users might fabricate or exaggerate their comments, as with other online analyses. As Hine (2015) argues, such authenticity issues apply to other methods (e.g., interviews and surveys) and, similar to other methods, our data on users' medical and health behaviours are self-reports. Likewise, users might be pretending to be someone else or posting pictures of other men. We did not observe any cases of impersonation, but users did openly post pictures of other men to both praise and mock them. Second, we sampled threads that appeared popular or engaging, but another approach would have been to randomly sample threads to provide a more representative picture of looksmaxxing conversations. Although this is a useful strategy, our approach avoids coding threads that receive low engagement.

4 | Findings

Our findings are presented in three themes. First, we detail the hegemonic masculine gaze of looksmaxxing as users analyse and critique each other's bodies as masculine projects. Second, we describe how users medicalise masculinity by connecting interventions to perceived masculine deficits. Third, despite being advertised as a self-improvement practice, we show how looksmaxxing demoralises and harms the health of men and boys.

4.1 | The Hegemonic Masculine Gaze of Looksmaxxing

We argue that looksmaxxing applies a hegemonic masculine gaze to men's bodies. Similar to other discussions of gaze (e.g., Foucault 1973), its object receives intimate observation and is tied to power relations. The specific power relations, in this case, are bound to lookism and hegemonic masculinity (Connell 1995) as men are pursuing bodies that they tie to the social benefits received by both physically attractive and dominant men. Members weaponise hegemonic masculine norms against each other, critiquing bodies and extensively diagnosing masculine imperfections.

To demonstrate how this gaze works, we first summarise a rating thread. In this thread, User 1 asks to be rated and uploads four photos showing his side, front and three-quarter profiles. Some users state that he is average-looking, has 'great potential' or is slightly above average. Others critique his appearance, stating only '[his] eyes save [him] from being a turbosubhuman', '[his] bone development brutally failed' and 'he has no mandible'. Some users rate him out of 10, suggesting scores of 1, 1.5, 5 or 6. Responding to his ratings, User 1 agrees that his 'side profile sucks' because of his 'downwarded [sic] grown maxilla' and suggests he might pursue surgery. In every thread we analysed, users were insulted, unfavourably compared to other men, and/or encouraged to harm themselves by at least one other user. These discussions demonstrate the hegemonic masculine gaze, as users are critiqued for their apparent masculine deficits, such as their young faces, weak chins or lack of musculature.

In some respects, these ratings are also degradation ceremonies (Garfinkel 1956; L. F. Monaghan 2021) focused on masculinity. Specifically, users have their bodies and ability to perform masculinity publicly critiqued by dozens or hundreds of other men, who often go into intimate detail about their perceived shortcomings or the hopelessness of their gendered performance. As with degradation ceremonies, rather than providing advice or feedback, these interactions can reduce someone's status as a man.

The hegemonic masculine gaze is also used to rank order and quantify attractiveness. Users most frequently rate each other using the PSL scale, an acronym for three manosphere forums (Puahate.com, Sluthate.com and Lookism.net), which are positioned to have developed 'objective' criteria for rating physical attractiveness. As User 2 suggests:

> [The PSL system] has its flaws... [but] there is in fact science behind what is considered attractive and what features are ideal. PSL ratings are a good general way to classify who comes closest to the scientific ideal.

On this scale, White men who are 9/10 or 10/10 are referred to as 'Chads', whereas those just below them are labelled 'Chadlytes'. Attractive men of colour are referred to with racist and hypersexualised caricatures, such as 'Tyrone' (Black men) or 'Chang' (Asian men). They are seen as subordinate to Chads but superior to average-looking men of all races (see also Halpin and Richard 2021). Men who are labelled as 'normies' or 'mid-tier normies' receive scores of approximately 5/10; however, being 'average' is not an accomplishment in this community, as average-looking men are seen as unlikely to have a heterosexual relationship or succeed in life. Men who score 3/10 or lower are 'subhuman' whose lives are 'over' and are often told they are 'too ugly' to become Chads through looksmaxxing, but that

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looksmaxxing might help them look 'normal'. Accordingly, ratings critically position men in relation to hegemonic ideals and users who fail to meet these ideals are seen as requiring interventions or are told to give up on their chances for happiness and success. As we will demonstrate, this gaze has consequences for health, as men and boys are insulted, encouraged to self-harm or told to pursue substantive medical interventions.

Users are often humiliated if they attempt to disagree with these 'objective' scores. After User 3 disagrees with his evaluations, User 4 states that it is 'hilarious' he thinks he's been underrated because he has 'literally 0 good features', whereas User 5 adds:

> You're just very ugly and any super NT [neurotypical] normie on the street would say the same. How can you be so delusional to look at your photos, post them and seriously think people will call you good looking while you look like that? You got fucked big time by nature. Your face looks grotesque, like a kid would draw in a comic book.

Through such policing, we suggest that members maintain the dominance of hegemonic expectations and subordinate men who do not meet such bodily ideals.

We argue that users subject themselves to this hegemonic masculine gaze for four reasons. First, users are seeking advice for how to become more sexually desirable, to increase their number of sexual partners, or to 'get a girlfriend' and participate in heterosexual sex (e.g., because 'girls only want to fuck Chads'). Second, and related to the first point, members position looksmaxxing as a means to align their bodies with hegemonic expectations so that their masculine bodily capital can increase. Third, the ratings provide users with a seemingly 'objective' measure of attractiveness. Aligned with the previous point, ratings are tied to improving hegemonic and sexual appeal, as specific practices or interventions (detailed below) can 'objectively' raise someone's score. Fourth, fellow users are seen as less critical than women, with User 6 noting that there is a 'huge difference between rating on toilet gaze and PSL autists'. Here, 'toilet' refers to women, with the suggestion being that women are harsher judges than men. Although this point seems counterintuitive, it might suggest that members position any form of heterosexual rejection as a worse experience than even the caustic critiques they receive from other men. Such ideas also buttress the red pill ideology of the manosphere by positioning women as hostile towards men.

We further suggest that men supply ratings to participate in hegemonic masculinity. By applying the hegemonic masculine gaze to other men's bodies, users can participate in hegemonic masculinity by reinforcing masculine ideals, subordinating other men, and degrading women—even if they struggle to achieve hegemonic ideals. Users are subject to harsh critiques, but the community also allows them to weaponise hegemonic masculinity against others to elevate—if only briefly—their standing. Here, the community facilitates a vicious circle wherein men are denied their masculinity, but a primary means of participating in hegemonic masculinity is to similarly deny it to other men.

4.2 | Self-Improvement, Body Image and Medicalisation of Masculinity

In this section, we demonstrate how the hegemonic masculine gaze translates into medicalised consequences for men, as men participating in looksmaxxing report pursuing interventions to increase their masculine appeal (see also Bell 2016; Rosenfeld and Faircloth 2006). More specifically, we analyse why and how men might pursue these interventions.

Although some users situate their appearance altering practices as means to simply look or feel better, they more typically position them as a means to appeal to women or to 'ascend' (i.e., have a romantic/sexual relationship). For instance, User 7 states he will 'never find love if [he] doesn't get uee [upper eyelid] filler, canthoplasty and orbital implants'. Likewise, User 8, tells another member who shares his pictures, 'you need a lip lift man. You need to fix them lips and you will ascend hard. Right now, you are like [in] trucel territory [i.e., a real incel], but with better lips you would be a PSL god for sure'. Others provide the user with similar forms of advice, ranging from jaw surgery to plucking his eyebrows, for the purposes of getting matches on Tinder. Whether such recommendations are trivial or consequential, they are often aimed at increasing sexual success.

Having detailed why men pursue interventions, the remainder of this theme describes how they report intervening on their bodies. Some men aim to change their appearance through cosmetic practices, such as User 9, who seeks to alter the fat distribution in his face by giving his eyes a regular ice bath. Other practices include skincare routines, exercise, dieting, eyebrow plucking, wardrobe improvement, or growing a beard to 'disguise' their 'weak chins'. Here, in contrast to Watson (2000), the users are not just seeking to 'be in shape' or have an 'everyday body', but to have an exceptional body that makes them desirable and places them above other men.

Although many of these interventions are relatively modest or common, other practices involve more substantial behavioural changes. Mewing is one popular looksmaxxing practice (Notopoulos 2023). Two orthodontists developed mewing, which involves pressing the tip of the tongue against the roof of the mouth. Users argue this technique will change their face over time, producing a more masculine jaw. Although there is no evidence that mewing works (Lee, Graves and Friedlander 2019), users nonetheless promote it:

> If you are like me and have been hard mewing for years without proper jaw posture, at first anything you do with the tongue will only further prevent the jaw from working as intended. Together, the jaw and the neck establish a solid frame within which the tongue functions. Pushing hard with the tongue without increasing mandibular engagement in proportion will cause the tongue to overstep the boundaries of its frame, leading to postural dysfunction.
>
> (User 10)

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We also observed a user who reported sleeping on his side, wearing a backpack and with his bed on a decline to ensure that he mews throughout his sleep. Although changing how the tongue is positioned might appear to be a relatively banal intervention, users report engaging in mewing to a potentially obsessive and harmful degree.

One of the most invasive behavioural modifications we observed was users discussing a practice called 'bonesmashing'. This practice involves deliberately hitting bones to create small injuries, resulting in the bone growing back stronger and more pronounced. Users suggest that bone smashing can produce a more masculine and defined jaw or orbital region. For example, after User 7 complains about his face looking too juvenile, another user jokes that he could always 'bonesmash' with User 7 replying, 'I do bone smash'. User 11 likewise states he has 'been practicing' bonesmashing with a massage gun for several months. We argue that this practice is tethered to hegemonic ideas about male beauty, both in terms of the aggressive name of the practice and that men are physically harming themselves to appear more manly. Furthermore, men who practice bonesmashing would experience health consequences (e.g., bleeding, bruising, and soft tissue damage) as they are explicitly aiming to damage their bones to shape their appearance.

Users also discuss improving their appearance through surgeries, such as leg lengthening, rhinoplasty, jaw surgeries, brow ridge shaving, and procedures to move fat or correct gynaecomastia. For instance, users discuss how leg lengthening surgeries, which involve breaking bones in the legs to encourage additional growth, are painful and require many months of recovery. However, users also argue that 'LL' (leg lengthening) is the 'only way' to gain height after puberty, with users suggesting their options are 'LL or death' or stating that only, '5′10 and 5′11 guys are good candidates for LL, anything under and you will just go from subhuman to subhuman with LL' (User 12). Here, interventions intersect with the hegemonic masculine gaze as only bodies that meet specific masculine criteria can use medicalisation to participate in hegemonic masculinity.

Surgeries are broadly positioned as a means of improving someone's PSL rating and/or a way to become more attractive to others. For example, User 13 suggests surgery to a fellow member:

> A long face isn't the worst thing, but your face is way too narrow to offset it. Some people look good with long faces, but they usually have wide faces to create harmony. Yours looks very thin and frail, you have really bad UEE (upper eyelid exposure), your mouth is thin, your nose is narrow (which isn't a good thing it looks thin and sickly, good looking noses tend to have wider bridges). [Y]our ears are very pronounced, your hairline is set very high, your jaw is very narrow. Not even surgeries are going to get you very far with that base but if you rule out surgeries entirely then you might as well give up and find another hobby, there's little to no self-improvement waiting for you...

Here, User 13 tells another member that being a Chad is out of reach, but to have any hope of being a man or 'normal' at all, he must pursue medical interventions. Likewise, User 1, who we introduced above, states in his rating thread that he needs 'too many' surgeries to 'actually ascend', including 'rhino[plasty]', surgery to correct his perceived 'gyno' (i.e., gynaecomastia) and leg lengthening to add to his 5′6 height. As he cannot currently afford all these surgeries, he plans to get 'gyno surgery', save for other procedures and move to South East Asia, because looksmaxxing and other manosphere communities assert that women of colour desire White men (Halpin and Richard 2021). In these cases, medicalisation does not confer hegemonic status but mitigates subordination.

Users do critically discuss the merits of surgery. They argue that leg lengthening is debilitating and might not work, whereas others suggest that 'too much' surgery can make someone 'look gay' or unnatural. In one exchange, User 7 states that he feels like he has ruined his life and become an incel by following the recommendations of the community (e.g., surgery). In response, User 14 offers some support:

> It's sad how they [the looksmaxxing community] warped your mind. You went from a nice looking kid to weird looking because of all those fillers... [User 20], I heavily recommend to reverse anything you've done to your face. Once you do so you'll [look] real nice no cap [no lie]. Also never do plastic surgery again, you were a good looking kid and never needed it... You seem like a nice kid who got fucked by PSL basement dwelling incel f*gs [slur for gay men] on this site. Keep working out, lose the fillers, if you didn't graduate HS get a GED and try to go to a college as far away from your hometown as possible... If you need someone to talk to I'd be happy to talk to you or help you out with anything you'd ever need.

Here, User 14 argues that User 7 has ruined his appearance and that 'too much' plastic surgery makes someone both ugly and unmanly. Instead, men must thread the needle of looksmaxxing, using medicine to enhance their masculinity, but receiving too much intervention—or having the appearance of having had too many interventions—is not masculine.

In relation to our first theme, we argue that the hegemonic masculine gaze works 'hand in glove' with the medicalisation of masculinity, as men pursue interventions to address perceived shortcomings in their gendered performance. Although this finding is contrary to Courtenay (2000a, 2000b) and others' arguments about hegemonic masculinity and men's avoidance of medicine, we also observed the policing of 'too much' intervention and the emasculation of men who were seen as obviously looksmaxxing (detailed above). Taken together, the community advocates a 'goldilocks' approach to intervention, wherein men need to looksmax to participate in hegemonic masculinity, but too much looksmaxxing is perceived as effeminate and violating hegemonic ideals. We further argue that, despite the community's apparently supportive aims, it leverages lookism and hegemonic

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ideals against men, such that they view their physical appearance as being responsible for the troubles in their lives. As a result, men might medicalise their bodies by pursuing costly and substantive interventions or engaging in harmful practices (e.g., mewing and bonesmashing) in an effort to change their lives by way of changing their appearance.

4.3 | From Self-Improvement to Masculine Demoralisation

Looksmaxxing is positioned as a self-improvement practice. In this section, we argue that looksmaxxing instead negatively impacts men's and boys' health and self-worth. First, we contend that users only provide approval when other men conform to narrow hegemonic masculine ideals. Second, instead of providing support, we argue that the community engages in masculine demoralisation of users, situating their manhood and lives as flawed because of their appearance. Here, looksmaxxing harms the health of men and boys, as the hegemonic masculine gaze is weaponised against them and their appearance becomes their only salient feature.

When looksmaxxers do receive compliments and support, it is when they conform to the hegemonic ideals of the community. For example, User 15 posts photos showing his self-proclaimed transformation from 'subhuman to Chad', outlining alterations to his weight, skin, hair, eyebrows, eyelashes and teeth. He further adds, 'I got a girlfriend from the begin[ning] of [my] looksmaxxing [un]til now and still love her so much'. Users praise his transformation, stating, 'high iq thread bro', 'very motivating' and 'looks good [you're a] legit Chad' with many users stating he 'mogs' [dominates] them. User 16 likewise receives approval after showing pictures and reporting that he changed his appearance with 'botox in masseters [the jaw], dying hair and brows, veneers, tanning, surgically correcting lazy eye, losing significant bodyfat and building muscle'. Many users label him a Chad, whereas User 17 states, 'great transformation bro. You worked very hard for this. Congrats'. As we argue, Users 15, 16 and others receive praise because their transitions take place in relatively confined bounds of success for the looksmaxxing community, as they are perceived as making their bodies more masculine, having sexual success, and raising their PSL score. Such praise also buttresses our medicalisation arguments above, as both users tie their sexual successes to major physical interventions.

However, as users pursue an ideal, approval is never universal and is instead blended with critique. For instance, one user suggests that User 15 is not a 'Chad' because his PSL score is 5 (or 'average'). User 18 posts a photo of a 'real' Chad beside User 16, stating that he is a 'HTN [high-tier normie] at the most. People saying chad [are] fucking n****** [a racial slur for Black people] IQ r****** [a slur for people with an intellectual disability]. You get mogged to oblivion by actual Chad'. In these cases, members use pictures of other men to weaponise the hegemonic masculine gaze, with even apparent 'success stories' being positioned as inferior to an idealised masculine body. In these arguments, men can never really 'make it' as 'real men' because they are always positioned as lacking in relation to a trait or in comparison to another man. Such undermining keeps attractive members aligned with other users, as they all share a common experience of being subordinated by more attractive men.

In contrast to the praise that some men receive, other men are emphatically denigrated. In these conversations, looksmaxxing resembles an echo chamber of masculine demoralisation rather than a self-improvement community. By masculine demoralisation, we mean that men are told that they deviate from accepted male beauty standards, that they have no hope of having a romantic/sexual relationship, and that they are failures as men. They are positioned as permanently subordinated (Connell 1995) and ostracised from the hegemonic masculinity that they are pursuing. As we argue, this masculine demoralisation does not untether men from hegemonic masculinity as they have only given up on their ability to meet hegemonic ideals rather than the importance of such ideals. For such men, looksmaxxing is not a path to hegemony but a means to appear 'normal' or 'human'.

Our data contains many examples of masculine demoralisation. For instance, in response to another member debating the feedback he received from the community, User 19 replies, 'no women would see you and feel something. Maybe if you're not acting like a child some users can help you to ascend otherwise just buy a rope [i.e., complete suicide]' (see also Daly and Laskovtsov 2022). Similarly, while reviewing photos of another man, User 20 states:

> Considering he is a healthy human being—I'd put him in the 2.25–2.5 area [on the PSL rating scale]. But looks wise—I've seen literal deformed, down syndrome, burn victims look better than op [original poster].

In a similar example, User 21—who states he is '5′11 with a semi-muscular build'—posts a photo of his face and asks to be rated. Many users respond with critiques:

> It's brutally over for you... sorry I can't imagine any girl staring at you other than for your goofy [looks].
>
> (User 22)

> You're ugly as shit how can you not see that?
>
> (User 23)

> You have various horrendous falios [i.e., the opposite of a feature with a halo effect, features that make you appear ugly].
>
> (User 24)

> Death tier mouth to nose ratio. Death tier philtrum to chin ratio. Death tier nose. Death tier philtrum length. Death tier eyes. Just give up.
>
> (User 25)

When User 21 pushes back against his rating, claiming that he gets attention from women, the others double down on their critiques. Many users comment that women are looking at him because they find him grotesque, whereas User 26 states, 'do

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you expect everyone to say that you are a Chadlite or [something]?' As the thread concludes, User 21 acknowledges that, 'I guess my lower part [of my face] is fucked, but I can still fix under eye area, eyebrow thickness [and] form, bodyfat [percentage] so my face bones are more prominent [and] hair, as you said'. In these and other cases, men who fall outside the perceived norms of hegemonic masculinity are subordinated and positioned as repulsive subhumans who are categorically unattractive to all women. As the first part of this theme demonstrates, such masculine demoralisation is common as even men who are perceived as attractive by some users are degraded and denigrated.

Masculine demoralisation is most clearly demonstrated when users encourage others to complete suicide. Although the forum prohibits spam, race-bating, and LBGTQ content, it does not prohibit discussions or encouragement of self-harm. We observed dozens of instances of suicide encouragement, with users typically telling other men 'it's over' [i.e., their lives and chances of having a relationship are over] and to 'rope' [i.e., complete suicide by hanging]. In other threads, users are encouraged to 'slit [their] wrists' (User 27), 'rope', (User 28) or 'jump in front of a train' (User 29).

In response to such demoralisation, users also state their desire to self-harm, with User 30 stating, 'this forum has me on the verge of roping'. After agreeing with other users that looksmaxxing procedures have ruined his face, User 7 states that the community, 'almost drove me to suicide'. After User 31 states that he is only 5′6, User 1 (who is also 5′6) tells him 'it's over' and he should contemplate suicide, with User 31 agreeing, 'I should rope. Over'. One teenage user's bio—which appears alongside every post he makes on the discussion board—states that he will 'rope' if he does not reach a height of 6′3 by the age of 20. As we argue, these examples evidence masculine demoralisation, as men are positioned as failing to be real men and, because of their inability to be real men, they are encouraged to commit suicide.

This demoralisation can be both symmetrical and asymmetrical. By symmetrical, we mean that men who demoralise others are also subject to demoralisation. For instance, after posting his photo, User 1 is encouraged to complete suicide, but he also encourages User 31 to similarly self-harm. By asymmetrical, we mean that some users demoralise other men without subjecting themselves to similar demoralisation and do not post their own pictures or describe their bodies. In some of these cases, users can be 'called out' to 'post proof' of their appearance, with failure to do so resulting in insults, assumptions that the user is ugly, or their critiques being ignored. Nonetheless, users can and do leverage the anonymity of online communities to critique other members without subjecting their own bodies to such evaluations.

Although looksmaxxing is positioned as a self-improvement practice, we argue that men instead weaponise the hegemonic masculine gaze against one another. All men are judged in relation to masculine ideals, with most men found to be painfully inadequate and excluded from masculinity. In this section, we demonstrated how the community produces tangible harms for members, as their self-worth is eviscerated and suicide is encouraged.

5 | Discussion

Our study analyzes looksmaxxing and develops concepts to understand the potential harms of self-improvement in the manosphere. As we argue, men participate in looksmaxxing because they position the community as providing objective information about their bodies and concrete advice about how to improve their looks. Self-improvement is not inherently harmful and these men seek to enhance their self-esteem, self-image and lives. However, we demonstrate that they are instead subjected to a hegemonic masculine gaze that caustically evaluates them in relation to an unachievable masculine ideal. After receiving such evaluations, men report engaging in substantial, expensive and potentially harmful health interventions. They are also subject to masculine demoralisation, which positions them as failed men and encourages self-harm. We have demonstrated that, while operating under the guise of self-improvement, looksmaxxing harms the health of men and boys.

Considerable research demonstrates how patriarchy and hegemonic masculinity have a negative impact on women (Collins 1990; Connell 1995). Research on the manosphere likewise indicates that these communities are harmful to women (Massanari 2017; Marwick and Caplan 2018; Van Valkenburgh 2021; Halpin 2022). Although men can receive benefits from hegemonic masculinity, our analysis aligns with previous studies that suggest that hegemonic masculinity and patriarchy also have negative consequences for men (e.g., Anderson 2005; Connell and Messerschmidt 2005; Courtenay 2000a, 2000b). Indeed, we suggest that looksmaxxing is an exemplar of how hegemonic masculinity can harm the men who endorse it, as the practice has clear consequences for the health of men and boys (e.g., suicidal ideation). As we demonstrate, looksmaxxing combines lookism and hegemonic masculinity to leverage a hegemonic masculine gaze that judges all men in relation to hegemonic ideals, finding that most men are found wanting, and positioning minute distances from perceived ideals as abject failures. Just as the male gaze is harmful for women, we argue this gaze can likewise harm men. Given that looksmaxxing is an increasingly popular practice (Notopoulos 2023), we suggest this is also a growing health and social issue that requires the attention of researchers, the public, and policymakers.

Lookism has several pernicious consequences (Monk, Esposito and Lee 2021), with research demonstrating that body image expectations can negatively impact the physical and mental health of both men and women (Monks et al. 2021). We build on this research by explaining how the looksmaxxing community's endorsement of lookism negatively affects community members. First, we argue that the community's endorsement of lookism medicalises men's bodies (see also Bell 2016; Rosenfeld and Faircloth 2006), as they translate men's issues (e.g., romantic troubles, employment troubles and social isolation) into personal troubles to be solved with health commodities and practices (e.g., surgeries and mewing). Second, we suggest that gendered performances amplify the consequences of lookism. Men are intimately critiqued and even slight deviations from ideals must be corrected or they will never be loved, happy, or 'real men'. As we suggest, how these communities 'do gender' compounds the negative impacts that lookism can have on men's body image and health. Furthermore, looksmaxxing

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might create anxieties that result in men pursuing medical services, particularly for-profit and elective services, to modify their gendered performance. Future research could examine if for-profit health/medical companies orient their services to looksmaxxing discourses.

Courtenay's (2000a) theory arguing that commitments to hegemonic masculinity damage men's health has received both critique and support (Quaglia 2020; Ratcliffe, Kanaan and Galdas 2023). Although our analysis of looksmaxxing supports the claim that an orientation to hegemonic masculinity can harm men's health, we contend that our findings substantially complicate Courtenay's (2000a) framework. Aligning with arguments that Courtenay's framework is overly simplified (Griffith 2024), we assert that looksmaxxing evidences a feedback loop between masculinity and health. First, to appear 'more masculine', men report engaging in health practices and soliciting medical services (e.g., mewing and leg lengthening). Second, these men are chasing an idealised masculinity and are subject to a hegemonic masculine gaze from other men. As a result, masculinity is a perpetually moving target and men report seeking additional interventions to make themselves more masculine. This is seen most clearly when users like 15 and 16 receive praise from some users, but have their masculine status denied by others. As a result of this feedback loop, men are positioned as pursuing perfection or accepting failure, as they can be caught chasing masculinity through medicalisation or give up on both medicalisation and their hopes of participating in hegemonic masculinity because of masculine demoralisation. As we have shown, both scenarios harm men's health. Although we have articulated this feedback loop using an online community, we suggest it broadly applies to situations wherein men discuss health (e.g., locker rooms, clubs and golf courses) or in 'male preserves' (Matthews 2016).

Courtenay's (2000a) framework is useful and influential; however, given our arguments above and prior research (e.g., Norman 2011; Quaglia 2020), we suggest the theory also needs to be advanced and revised. First, we argue that men's commitments to hegemonic masculinity can compel health behaviours and interventions if such interventions are seen as conferring masculinity (see also Denham 2008). We further suggest that this is particularly salient for men who see their engagement with masculinity as precarious. Second, we argue that men can weaponise hegemonic masculinity to incite other men to engage in health/medical practices and that such incitement can have negative ramifications for physical and mental health. Third, we assert that practices that are not consistent with hegemonic masculinity (e.g., dieting) are best conceptualised as a masculine backstage (Goffman 1956), wherein men coordinate with other men in ways that might appear to deviate from hegemonic expectations (e.g., practices that might be seen as 'gay') to present an effortless, appealing and hegemonic front. Fourth, we suggest that some men avoid health behaviours not because they are performing hegemonic masculinity but, because of masculine demoralisation, they see themselves as 'ruled out' of hegemonic presentations. Such a mismatch between pursued gender health performance and how others interpret that performance is potentially harmful to physical and mental health. In this sense, the relationship between men's health and gender presentation is bidirectional, as perceived health status can impact how they see and perform gender and vice versa. Here, we further suggest that lookism and its potential effects on men's health and gendered performances require additional analyses.

On the surface, the looksmaxxing community and similar male-oriented self-improvement communities appear to raise a pointed dilemma; on one hand, these communities are harmful to men and it seems reasonable to develop health policy interventions that provide users with support. On the other hand, the users are also explicitly racist, homophobic, and ubiquitously demeaning women. From this perspective, offering care and support to men who harm others might appear counterintuitive or harmful. We suggest this is a false debate and a misdiagnosis of the issue. Rather than solely focussing on the outcomes of the looksmaxxing community (e.g., misogyny and self-harm), we also encourage a focus on the common causes of these harms, namely, the community's commitment to male supremacy and hegemonic masculinity. As we argue, these commitments facilitate the degradation of both men and women. Rather than doubling down on ephemeral masculine ideals that have arguably failed and excluded them, we suggest that a means to both disrupt the misogyny and alleviate the harm of such communities is to encourage men to imagine new forms of masculinity that accept them as they are, rather than ask them to suffer for something that they might never become.

Author Contributions

Michael Halpin: conceptualization (lead), formal analysis (lead), funding acquisition (lead), methodology (lead), supervision (lead), writing – original draft (equal), writing – review and editing (lead). **Meghan Gosse:** data curation (lead), formal analysis (supporting), writing – original draft (equal). **Katharine Yeo:** writing – original draft (supporting). **Ingrid Handlovsky:** writing – review and editing (supporting). **Finlay Maguire:** funding acquisition (supporting).

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Ethics Statement

This research was approved by the Dalhousie University Research Ethics Board.

Conflicts of Interest

The authors declare no conflicts of interest.

Data Availability Statement

Data is available upon request.

References

Anderson, E. 2005. *In the Game: Gay Athletes and the Cult of Masculinity*. State University of New York Press. https://hdl.handle.net/2027/heb33313.0001.001.

Bell, A. V. 2016. "'I Don't Consider a Cup Performance; I Consider It a Test': Masculinity and the Medicalisation of Infertility." *Sociology of*

14679566, 2025, 3, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/1467-9566.70015 by Pontificia Universidad Catolica De Chile, Wiley Online Library on [13/11/2026]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

Health & Illness 38, no. 5: 706–720. https://doi.org/10.1111/1467-9566.12395.

Collins, P. H. 1990. *Black Feminist Thought: Knowledge, Consciousness and the Politics of Empowerment*. New York, NY: Routledge.

Connell, R. 1995. *Masculinities*. Berkeley: University of California Press.

Connell, R. and J. W. Messerschmidt. 2005. "Hegemonic Masculinity: Rethinking the Concept." *Gender & Society* 19, no. 6: 829–859. https://doi.org/10.1177/0891243205278639.

Courtenay, W. H. 2000a. "'Constructions of Masculinity and Their Influence on Men's Well-Being: A Theory of Gender and Health." *Social Science & Medicine* 50, no. 10: 1385–1405.

Courtenay, W. H. 2000b. "Engendering Health: A Social Constructionist Examination of Men's Health Beliefs and Behaviors." *Psychology of Men and Masculinity* 1, no. 1: 4–15. https://doi.org/10.1037/1524-9220.1.1.4.

Dahlgren, C. L., C. Sundgot-Borgen, I. L. Kvalem, A. L. Wennersberg and L. Wisting. 2024. "Further Evidence of the Association Between Social Media Use, Eating Disorder Pathology and Appearance Ideals and Pressure: A Cross-Sectional Study in Norwegian Adolescents." *Journal of Eating Disorders* 12, no. 1: 34. https://doi.org/10.1186/s40337-024-00992-3.

Daly, S. E., and A. Laskovtsov. 2022. ""Goodbye, My Friendcels": An Analysis of Incel Suicide Posts." *Journal of Qualitative Criminal Justice & Criminology* 11, no. 1: 1–37.

Denham, B. E. 2008. "Masculinities in Hardcore Bodybuilding." *Men and Masculinities* 11, no. 2: 234–242. https://doi.org/10.1177/1097184X07304809.

Dignam, P. A. and D. A. Rohlinger. 2019. "Misogynistic Men Online: How the Red Pill Helped Elect Trump." *Signs: Journal of Women in Culture and Society* 44, no. 3: 589–612. https://doi.org/10.1086/701155.

Dion, K., E. Berscheid and E. Walster. 1972. "What Is Beautiful Is Good." *Journal of Personality and Social Psychology* 24, no. 3: 285–290. https://doi.org/10.1037/h0033731.

Filice, E., A. Raffoul, S. B. Meyer and E. Neiterman. 2020. "The Impact of Social Media on Body Image Perceptions and Bodily Practices Among Gay, Bisexual and Other Men Who Have Sex With Men: A Critical Review of the Literature and Extension of Theory." *Sex Roles* 82, no. 7: 387–410. https://doi.org/10.1007/s11199-019-01063-7.

Fixsen, A., D. Ridge, O. Ponsford, M. Holder and G. Saran. 2023. "Battles over 'Unruly Bodies': Practitioners' Interpretations of Eating Disorders and the Utility of Psychiatric Labelling." *Sociology of Health & Illness* 45, no. 3: 560–579. https://doi.org/10.1111/1467-9566.13601.

Foucault, M. 1973. *The Birth of the Clinic: An Archaeology of Medical Perception*. London, UK: Tavistock.

Galdas, P. M., F. Cheater and P. Marshall. 2005. "Men and Health Help-Seeking Behaviour: Literature Review." *Journal of Advanced Nursing* 49, no. 6: 616–623. https://doi.org/10.1111/j.1365-2648.2004.03331.x.

Garfinkel, H. 1956. "Conditions of Successful Degradation Ceremonies." *American Journal of Sociology* 61, no. 5: 420–424. https://doi.org/10.1086/221800.

Goffman, E. 1956. *The Presentation of Self in Everyday Life*. New York: Doubleday.

Gosse, M., M. Halpin and F. Maguire. 2024. "Stochastic Gender-Based Violence: How Incels Justify and Encourage Sexualized Violence Against Women." *Violence Against Women*: 10778012241279816. https://doi.org/10.1177/10778012241279816.

Griffith, D. M. 2024. "Gender Health Equity: The Case for Including Men's Health." *Social Science & Medicine* 351: 116863. https://doi.org/10.1016/j.socscimed.2024.116863.

Grogan, S. and H. Richards. 2002. "Body Image: Focus Groups With Boys and Men." *Men and Masculinities* 4, no. 3: 219–232. https://doi.org/10.1177/1097184X02004003001.

Halpin, M. 2022. "Weaponized Subordination: How Incels Discredit Themselves to Degrade Women." *Gender & Society* 36, no. 6: 813–837. https://doi.org/10.1177/08912432221128545.

Halpin, M. and N. Richard. 2021. "An Invitation to Analytic Abduction." *Methods in Psychology* 5: 100052. https://doi.org/10.1016/j.metip.2021.100052.

Halpin, M., N. Richard, K. Preston, M. Gosse and F. Maguire. 2025. "Men Who Hate Women: The Misogyny of Involuntarily Celibate Men." *New Media & Society* 27, no. 1: 424–442. https://doi.org/10.1177/14614448231176777.

Hine, C. 2015. *Ethnography for the Internet: Embedded, Embodied and Everyday*. London, UK: Taylor & Francis.

Johnson, J. L., J. L. Oliffe, M. T. Kelly, P. Galdas and J. S. Ogrodniczuk. 2012. "Men's Discourses of Help-Seeking in the Context of Depression." *Sociology of Health & Illness* 34, no. 3: 345–361. https://doi.org/10.1111/j.1467-9566.2011.01372.x.

Jordan-Young, R. M. and K. Karkazis. 2019. *Testosterone: An Unauthorized Biography*. Cambridge, MA: Harvard University Press.

Lee, U. K., L. L. Graves and A. H. Friedlander. 2019. "Mewing: Social Media's Alternative to Orthognathic Surgery?" *Journal of Oral and Maxillofacial Surgery* 77, no. 9: 1743–1744. https://doi.org/10.1016/j.joms.2019.03.024.

Marwick, A. E. and R. Caplan. 2018. "Drinking Male Tears: Language, the Manosphere and Networked Harassment." *Feminist Media Studies* 18, no. 4: 543–559. https://doi.org/10.1080/14680777.2018.1450568.

Maslen, S. and D. Lupton. 2019. "'Keeping It Real': Women's Enactments of Lay Health Knowledges and Expertise on Facebook." *Sociology of Health & Illness* 41, no. 8: 1637–1651. https://doi.org/10.1111/1467-9566.12982.

Massanari, A. 2017. "'#Gamergate and the Fappening: How Reddit's Algorithm, Governance and Culture Support Toxic Technocultures." *New Media & Society* 19, no. 3: 329–346. https://doi.org/10.1177/1461444815608807.

Matthews, C. R. 2016. "The Tyranny of the Male Preserve." *Gender & Society* 30, no. 2: 312–333. https://doi.org/10.1177/0891243215620557.

Monaghan, L. 1999. "Challenging Medicine? Bodybuilding, Drugs and Risk." *Sociology of Health & Illness* 21, no. 6: 707–734. https://doi.org/10.1111/1467-9566.00180.

Monaghan, L. F. 2021. "Degrading Bodies in Pandemic Times: Politicizing Cruelty During the COVID-19 and Obesity Crises." *Journal of Communication Inquiry* 49, no. 1: 01968599211043403. https://doi.org/10.1177/01968599211043403.

Monk, E. P., M. H. Esposito and H. Lee. 2021. "Beholding Inequality: Race, Gender and Returns to Physical Attractiveness in the United States." *American Journal of Sociology* 127, no. 1: 194–241. https://doi.org/10.1086/715141.

Monks, H., L. Costello, J. Dare and E. Reid Boyd. 2021. "'We're Continually Comparing Ourselves to Something': Navigating Body Image, Media and Social Media Ideals at the Nexus of Appearance, Health and Wellness." *Sex Roles* 84, no. 3: 221–237. https://doi.org/10.1007/s11199-020-01162-w.

Norman, M. E. 2011. "Embodying the Double-Bind of Masculinity: Young Men and Discourses of Normalcy, Health, Heterosexuality and Individualism." *Men and Masculinities* 14, no. 4: 430–449. https://doi.org/10.1177/1097184X11409360.

Notopoulos. 2023. "'Looksmaxxing' Is the New TikTok Trend for Young Men Who Want to Be Hot." *Yahoo Sports*. https://ca.news.yahoo.com/looksmaxxing-tiktok-trend-young-men-160728175.html.

Perloff, R. M. 2014. "Social Media Effects on Young Women's Body Image Concerns: Theoretical Perspectives and an Agenda for Research." *Sex Roles* 71, no. 11: 363–377. https://doi.org/10.1007/s11199-014-0384-6.

14679566, 2025, 3, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/1467-9566.70015 by Pontificia Universidad Catolica De Chile, Wiley Online Library on [13/11/2026]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License

Prichard, I., E. Kavanagh, K. E. Mulgrew, M. S. Lim and M. Tiggemann. 2020. "The Effect of Instagram #fitspiration Images on Young Women's Mood, Body Image and Exercise Behaviour." *Body Image* 33: 1–6. https://doi.org/10.1016/j.bodyim.2020.02.002.

Quaglia, V. 2020. "Men, Masculinities and Diabetes: 'Doing Gender' in Italian Men's Narratives of Chronic Illness." *Sociology of Health & Illness* 42, no. 8: 1902–1917. https://doi.org/10.1111/1467-9566.13176.

Ratcliffe, J., M. Kanaan and P. Galdas. 2023. "Reconceptualising Men's Loneliness: An Interpretivist Interview Study of UK-Based Men." *Social Science & Medicine* 332: 116129. https://doi.org/10.1016/j.socscimed.2023.116129.

River, J. and M. Flood. 2021. "Masculinities, Emotions and Men's Suicide." *Sociology of Health & Illness* 43, no. 4: 910–927. https://doi.org/10.1111/1467-9566.13257.

Rosenfeld, D. and C. A. Faircloth. 2006. *Medicalized Masculinities*. Philadelphia, PA: Temple University Press. https://www.jstor.org/stable/j.ctt14bt2t2.

Simpson, P. and J. Adams. 2019. "A Structured Review and Critical Analysis of Male Perceptions of the Penis: A Comparison Between Heterosexual Men and Men Who Have Sex With Men (MSM)." *Men and Masculinities* 22, no. 4: 658–693. https://doi.org/10.1177/1097184X17715054.

Simpson, P. and M. Richards. 2019. "Reflexivity Denied? The Emotional and Health-Seeking Resources of Men Facing Disadvantage." *Sociology of Health & Illness* 41, no. 5: 900–916. https://doi.org/10.1111/1467-9566.12869.

Solea, A. I. and L. Sugiura. 2023. "Mainstreaming the Blackpill: Understanding the Incel Community on TikTok." *European Journal on Criminal Policy and Research* 29, no. 3: 311–336. https://doi.org/10.1007/s10610-023-09559-5.

Springer, K. W. and D. M. Mouzon. 2011. "'Macho Men' and Preventive Health Care: Implications for Older Men in Different Social Classes." *Journal of Health and Social Behavior* 52, no. 2: 212–227. https://doi.org/10.1177/0022146510393972.

Tavory, I. and S. Timmermans. 2014. *Abductive Analysis: Theorizing Qualitative Research*. Chicago, IL: University of Chicago Press.

Van Valkenburgh, S. P. 2021. "Digesting the Red Pill: Masculinity and Neoliberalism in the Manosphere." *Men and Masculinities* 24, no. 1: 1097184X18816118. https://doi.org/10.1177/1097184X18816118.

Watson, J. 2000. *Male Bodies: Health, Culture and Identity*. Buckingham: Open University Press.

14679566, 2025, 3, Downloaded from https://onlinelibrary.wiley.com/doi/10.1111/1467-9566.70015 by Pontificia Universidad Catolica De Chile, Wiley Online Library on [13/11/2026]. See the Terms and Conditions (https://onlinelibrary.wiley.com/terms-and-conditions) on Wiley Online Library for rules of use; OA articles are governed by the applicable Creative Commons License